

HOUSE No. 2199

The Commonwealth of Massachusetts

PRESENTED BY:

Marjorie C. Decker

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act ending unnecessary hospitalizations.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>Marjorie C. Decker</i>	<i>25th Middlesex</i>	<i>1/15/2025</i>
<i>Estela A. Reyes</i>	<i>4th Essex</i>	<i>5/28/2025</i>

HOUSE No. 2199

By Representative Decker of Cambridge, a petition (accompanied by bill, House, No. 2199) of Marjorie C. Decker relative to the involuntary hospitalization. Mental Health, Substance Use and Recovery.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act ending unnecessary hospitalizations.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Section 1 of Chapter 123 of the General Laws is hereby amended by
2 inserting the following sentence after the second sentence:-

3 “Community Alternatives,” include but are not limited, to voluntary services provided
4 through Mobile Crisis Intervention (MCI), Behavioral Health Urgent Care, and Community
5 Crisis Stabilization (CCS) programs and other voluntary services at Community Behavioral
6 Health Centers; the MassHealth BH Urgent Care programs, Behavioral Health Helpline, 988
7 and peer-run programs.

8 SECTION 2. Section 12 of Chapter 123 of the General Laws is hereby amended by
9 striking the first paragraph in subsection (a) and inserting the following paragraph:-

10 (a) Prior to applying for involuntary detention pursuant to this section, a professional or
11 police officer authorized to apply for such detention must first determine that there is no
12 community alternative appropriate for the person. If the person is appropriate for and consents to

the community alternative, the professional or police officer shall arrange for transport, via ambulance or otherwise, to the community alternative. A physician who is licensed pursuant to section 2 of chapter 112, an advanced practice registered nurse authorized to practice as such under regulations promulgated pursuant to section 80B of said chapter 112, a qualified psychologist licensed pursuant to sections 118 to 129, inclusive, of said chapter 112 or a licensed independent clinical social worker licensed pursuant to sections 130 to 137, inclusive, of said chapter 112 who, after examining a person and after determining that there is no appropriate community alternative to involuntary hospitalization, has reason to believe that failure to hospitalize such person would create a likelihood of serious harm by reason of mental illness may restrain or authorize the restraint of such person and apply for the hospitalization of such person for a 3-day period at a public facility or at a private facility authorized for such purposes by the department. If an examination is not possible because of the emergency nature of the case and because of the refusal of the person to consent to such examination, the physician, qualified psychologist, qualified advanced practice registered nurse or licensed independent clinical social worker on the basis of the facts and circumstances may, after determining that there is no appropriate community alternative to involuntary hospitalization, determine that involuntary hospitalization is necessary and may therefore apply. In an emergency situation, if a physician, qualified psychologist, qualified advanced practice registered nurse or licensed independent clinical social worker is not available, a police officer who, after determining that there is no appropriate community alternative to involuntary hospitalization, believes that failure to hospitalize a person would create a likelihood of serious harm by reason of mental illness may restrain such person and apply for the hospitalization of such person for a 3-day period at a public facility or a private facility authorized for such purpose by the department. An application

for hospitalization shall state the community alternatives to involuntary hospitalization determined to be inappropriate, the reasons for the restraint of such person and any other relevant information that may assist the admitting physician or qualified advanced practice registered nurse. Whenever practicable, prior to transporting such person, the applicant shall telephone or otherwise communicate with a facility to describe the circumstances and known clinical history and to determine whether the facility is the proper facility to receive such person and to give notice of any restraint to be used and to determine whether such restraint is necessary.

SECTION 3. Section 12 of Chapter 123 of the General Laws is hereby amended adding after subsection (e) the following subsection:-

(f) The department shall collect information regarding all applications filed pursuant to this section, including the number of applications and other such information as may be relevant, including, but not limited to, information on the age, gender identity, race, ethnicity, insurance status, and diagnosis of individuals subject to an application. The department shall annually, not later than July 31, report to the house and senate committees on ways and means, joint committee on public health and the joint committee on mental health, substance use and recovery the number of applications pursuant to said section 12, other information as may be relevant, and any actions the department has taken in response to the information it has received, including any licensing actions.